New Somerset Hospital
Department of Anaesthesia

PATIENT STICKER
Name .....
Folder no. .....
Date of birth .....

REGIONAL ANAESTHESIA CONSENT — Truncal & Fascial Plane Block

1. PLANNED PROCEDURE & BLOCK

Procedure ..... Side: Left Right Bilateral

Block(s): ☐ Paravertebral ☐ Erector spinae (ESP) ☐ Intercostal ☐ Serratus anterior ☐ PECS I / II
☐ Parasternal ☐ TAP ☐ Rectus sheath ☐ Quadratus lumborum ☐ Ilioinguinal / IH
Other .....

Planned with: ☐ Block alone ☐ Sedation ☐ GA ☐ Spinal ☐ Site marked

Patient has capacity to consent: Yes No If no, consent given by (name / relationship) .....

☐ 2. BENEFITS DISCUSSED

why a block is recommended for this patient

- Excellent pain relief at the operation site (often 8–24 h)
- Rib fractures / chest surgery: deep breathing, coughing & physio easier → fewer chest infections
- Earlier eating, drinking, mobilising and discharge
- Less opioid needed: less nausea, vomiting, drowsiness, itch, constipation
- Alternative to epidural (e.g. on anticoagulants) with fewer side effects: hypotension, urinary retention, leg weakness

Other benefit .....

3. RISKS DISCUSSED

most patients have no complications · details and figures in the patient leaflet

☐ **General — all nerve and plane blocks**

Common (1 in 10–100)	Prolonged numbness / weakness (expected 8–24 h) · Partial or failed block · Rebound pain as the block wears off · Bruising / tenderness · Temporary tingling or numbness
Uncommon (1 in 100–1 000)	Bleeding / haematoma
Rare (1 in 1 000–10 000)	Local anaesthetic toxicity (fits, heart rhythm) · Permanent nerve damage (> 6 months)
Very rare (< 1 in 10 000)	Infection · Severe allergic reaction

☐ **Block-specific**

Common (1 in 10–100)	Visceral pain not covered · Hypotension / epidural spread
Uncommon (1 in 100–1 000)	Pneumothorax (collapsed lung) · Leg weakness / numbness
Rare (1 in 1 000–10 000)	LA toxicity with bilateral blocks
Very rare (< 1 in 10 000)	Bowel or organ puncture

Other risk discussed .....

4. ALTERNATIVES DISCUSSED

- ☐ Oral / IV analgesia (incl. opioids or PCA)
- ☐ Local anaesthetic infiltration by surgeon
- ☐ General anaesthetic without a block
- ☐ A different block

☐ 5. AFTERCARE ADVISED

- Regular analgesia before the block wears off
- Report breathlessness or chest pain (pneumothorax)
- Assisted first mobilisation
- Report numbness / weakness lasting > 48 h

6. CONFIRMATION

Right to refuse explained (no effect on surgery or care): Yes No ☐ Questions answered

Patient leaflet: QR shown Printed copy Declined

Interpreter: Not needed Used Language ..... Name .....

Patient gives consent for the block(s) above: YES NO

Patient leaflet

Doctor ..... MP no. ..... Signature .....

Date ..... Time .....

Patient leaflet: nsh-anaesthesia.github.io/Documents/leaflets/regional-truncal.html

DRAFT

SASA/SASRA Guidelines 2016 · McCombe & Bogod, Anaesthesia 2021 · frequencies approximate